TRIAGE

Triage is the process of determining the priority of patients' treatments by the severity of their condition or likelihood of recovery with and without treatment.

In the Emergency department, sequence of treatment of patients is determined by patient's triage category and not first cum first serve basis.

In case of large number of patients and longer waiting times, **periodic reassessment & re-triaging** of patients is important.

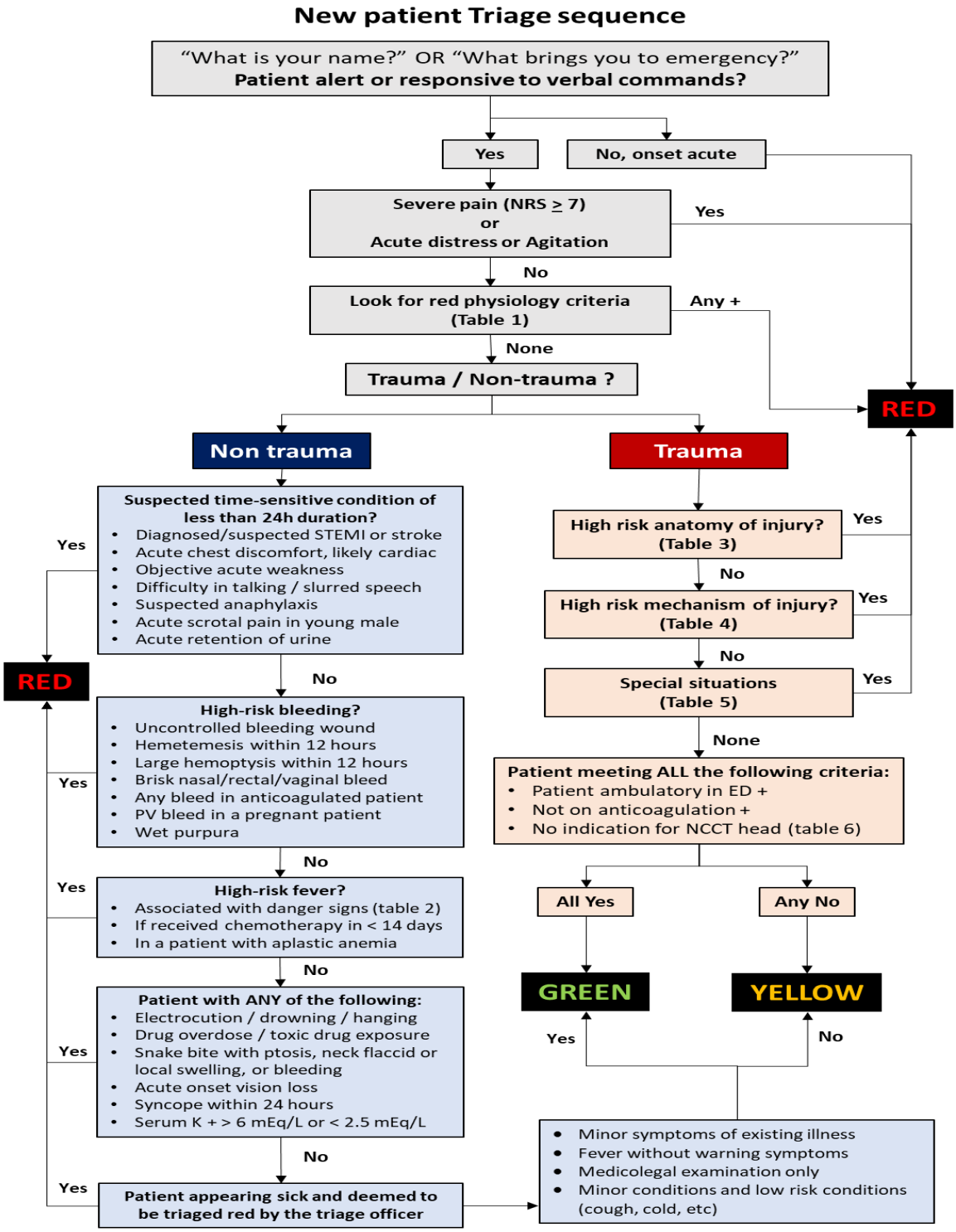

Table 1: RED Physiology criteria *(Triage red if any one of these mentioned vital criteria is present on the assessment)*			
A	• Stridor • Drooling of saliva • Noisy breathing • Angioedema of face	**C**	• Cardiac arrest • Pulse rate < 50 or > 120/min (without fever) • SBP < 90 or > 180 mmHg • DBP < 60 or DBP > 110 mmHg • Ongoing exsanguinating / major Hemorrhage
B	• Talking only in incomplete sentences • RR < 10/min or > 22/min • SpO2 < 90 %	**D**	• Unresponsive • Altered sensorium (Less than 'alert' on AVPU scale, or GCS ≤ 12) • Active seizures

Table 2: Danger signs in a patient with acute fever

- Bleeding from any site, or petechiae
- Jaundice / Icterus
- Recurrent vomiting
- Severe abdominal pain
- Altered sensorium / confusion / seizure
- Immunocompromised state
- Recent major surgical procedure (within 7 days)

Table 3: High-risk anatomy of injury

- C-spine injury with Single Breath Count < 15
- Visible neck swelling
- Major vascular injury
- **Chest trauma with:**
  - Surgical emphysema/
  - CCT positive/
  - Seat belt sign/
  - Flail chest
- Pelvic fracture (PCT +)
- Any new paralysis

Table 4: High-risk mechanisms of injury

- Gunshot wound
- Stab injury in torso
- Crush injury
- Amputation proximal to wrist or ankle
- Open fractures excluding hand/feet
- Fall from > 3 times patient's height
- Fall from > 5 stairs
- High-speed head-on collision of 2 vehicles
- Co-passenger death in Road traffic crash
- Ejection from 4-wheeler
- Railway track injuries
- Run over by vehicle
- Chemical splash in the eye

Table 5: Special situation in injury

- Pregnancy > 14 weeks
- Age > 65 years
- Patient with bleeding disorder
- Burns > 20% BSA, or facial burns

Table 6: Indications for NCCT head in trauma

- Vomiting > 2 times
- Severe headache
- Age > 65 years
- Focal neurologic deficits
- GCS < 13 on initial assessment
- GCS < 15 after 2 hours of injury
- Features of base of skull fracture
- Dangerous mechanism of injury (Table 4)
- Coagulopathy / patient on anticoagulants
- Amnesia > 30 min
- LOC > 5 min
- Seizures